Utah Department of Commerce
Office of Professional Licensure Review

2025 Periodic Review

Hearing Instrument Specialist

Published January 2026

Executive Summary

Background

Hearing instrument specialists (HISs) are hearing care professionals that are trained to evaluate common types of hearing loss in adults and dispense hearing aids. HISs practice in health and personal care retailers, as well as a variety of standard healthcare settings.

All states in the U.S. regulate HIS practice, and most (48) do so through licensure. In general, states require: completing an internship period while doing distance learning, passing a state law and rule examination, and passing written and practical examinations. However, Utah is the only state that requires applicants to obtain the NBC-HIS board certification for full licensure, which is a higher exam standard and requires at least two years of full-time dispensing experience.

Regulatory Model Recommendation: *Shift the regulatory model from licensure to mandatory certification*

- The potential for harm is generally low and mitigated by existing oversight.
- However, the potential lifetime effects of poor care mean provider competence is important, prompting the recommendation of mandatory certification.
- Mandatory certification entails reducing the administrative burden of regulation through:
    - Allowing 'once and done' certification; no renewal with DOPL
    - Moving from a single CE requirement to three options: 1) CEs or 2) national certification or 3) minimum number of working hours without a lapse
- The shift to mandatory certification would not change:
    - The need for specific entry requirements (education and training)
    - Background checks (moving to continuous FBI RapBack system over time)
    - DOPL oversight via investigation, discipline, and enforcement (e.g., DOPL's ability to remove an individual from the profession)

Recommended Regulatory Adjustments:

1) *Simplify the pathway to become a HIS by removing the NBC-HIS requirement and reducing the practice period as entry requirements*
    - The NBC-HIS board certification was determined to be overly burdensome as an entry-level requirement–the IHS/ILE exams are sufficient.
    - Reducing the practice period (from two years to six months) means sufficiently competent practitioners can enter full practice earlier, contributing to greater access.
2) *Allow audiologists to supervise HIS interns*
    - Audiologists are highly qualified and trained to prescribe and dispense hearing instruments.
    - This change would expand training opportunities for HIS interns.

Context

Consistent with its legislative mandate,[1] the Office of Professional Licensure Review (OPLR) reviewed Utah's licensing laws for hearing instrument specialist practitioners. The review evaluated how well current regulations:

1. Protect the public from present and consequential physical and financial harm
2. Balance public and practitioner access to the occupation
3. Limit the economic impact of regulation on consumers, practitioners and the state[2]

OPLR's research for this review included analysis of Utah's current laws and rules, licensing and complaint data from the Division of Professional Licensing (DOPL), academic literature, regulations in other states, and other secondary analyses. OPLR also conducted interviews with prominent stakeholders. See Appendix 1 for more information.

Background

Profession Overview

A hearing instrument specialist (HIS) is a hearing care professional that is trained to evaluate common types of hearing loss in adults and dispense hearing aids.[3] HISs are distinct from audiologists in several ways, specifically with regard to their education, training, experience, and scope of practice.[4] The practice of a HIS is relatively limited and focused on testing hearing loss in adults and prescribing appropriate hearing instruments. Conversely, an audiologist may treat and diagnose patients of all ages, not just adults, and their scope of practice encompasses all aspects of hearing, vestibular function,[5] and disorders of hearing, rather than just hearing loss.[6] This difference in scope reflects the differences in education and training between an audiologist and HIS, with audiologists having a doctorate degree and extensive clinical experience.

A HIS practices in a variety of standard healthcare settings, such as private or group practice and ambulatory healthcare services.[7] They also work extensively in health and personal care retailers,[8] which include major U.S. retailers with hearing centers, like Costco or Sam's Club.

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[1] UCA 13-1b-203(2)
[2] UCA 13-1b
[3] Definition of a hearing instrument specialist was derived from: What is a hearing instrument specialist?
[4] UCA 58-46a-102
[5] The vestibular system includes inner ear and brain structures that help maintain balance.
[6] UCA 58-41
[7] U.S. Bureau of Labor Statistics; Ambulatory Health Care Services provide healthcare services directly or indirectly to ambulatory patients and do not usually provide inpatient services. These include offices of physicians and other health practitioners.
[8] U.S. Bureau of Labor Statistics; Health and Personal Care Retailers is defined as retailers of health and personal care merchandise from fixed point-of-sale locations. They are characterized principally by the products they retail, and some health and personal care stores may have specialized staff trained in dealing with the products. Staff may include pharmacists, opticians, and other professionals engaged in retailing, advising customers, and/or fitting the product sold to the customer's needs.

Profession in Utah

In Utah, the practice of a HIS includes testing the hearing of an adult (18 years and older) for the sole purpose of determining whether hearing loss will be improved by the use of a hearing instrument, and if justified, prescribing an appropriate hearing instrument. Additionally, a HIS can make impressions or earmolds for the purpose of selling, fitting, and professionally placing a hearing instrument, and they evaluate the effectiveness of and adjust the settings on a hearing instrument.[9]

Utah also licenses HIS interns. HIS interns practice under the supervision of a HIS as they obtain the required education and experience. HIS interns have the same scope of practice as a HIS, but they must practice under the supervision of a HIS as they develop the skills required to practice in the profession.

In Utah, there are 136 HISs and 39 HIS interns (See Appendix 2.1).

To practice as a HIS in Utah, an individual must obtain and maintain a license through the Division of Professional Licensing (DOPL) within the Utah Department of Commerce. The requirements for licensure via the workplace training pathway include:

- Passing the Utah Law and Rules Examination for Hearing Instrument Specialists;
- Passing the International Hearing Society (IHS) practical examination;
- Passing the International Licensing Examination for Hearing Instrument Professionals (ILE) written examination administered by the IHS
- Holding a board certification by the National Board for Certification-Hearing Instrument Sciences (NBC-HIS), which requires the passage of the National Competency Exam (NCE) and a minimum of two years of full-time dispensing experience.[10]

There is also an academic training model; however, this pathway is relatively rare, as only two schools offer programs in the U.S.[11]

For a detailed breakdown on the process to become a HIS, please see Appendix 2.2.

Similarly, to practice as a hearing instrument specialist intern in Utah, an individual must obtain a license. The requirements for licensure as an intern include passing the Utah Law and Rules Examination for Hearing Instrument Specialists and providing evidence that the applicant will practice as an intern only under the supervision of a supervising HIS.[12] An intern must practice under the direct supervision of a HIS unless s/he receives a passing score on the IHS practical examination and completes the National Institute for Hearing Instrument Studies education and

[9] UCA 58-46a-102(5)
[10] UCA 58-46a-302(1)
[11] The two programs in the US include Spokane Falls Community College and the College of DuPage. Additionally, there are several academic programs in Canada.
[12] UCA 58-46a-302(2)

examination program,[13] in which case s/he may practice under indirect supervision.

Approaches in Other Jurisdictions

All states regulate HIS practice, and most (48) do so through licensure. The exceptions are, New York, Pennsylvania, and the District of Columbia, which regulate the profession through registration. In general, the requirements include completing supervised and/or unsupervised practice, passing a state law and rule exam, taking the IHS Distance Learning course, and passing written and practical examinations. However, states take different approaches to the pathway required to achieve licensure. These differences include variations on the time requirements for supervised practice, the written and practical examination provider (e.g., IHS, ILE, NBC-HIS, or state), and the requirement to be NBC-HIS board certified.

Utah is unique in that it is one of the only states that requires applicants to obtain the NBC-HIS board certification for full licensure as a HIS.[14] This includes obtaining a minimum of two years of full-time dispensing experience to be eligible to take the NBC-HIS certifying exam.[15] The required two-year process in Utah is longer than the requirements in other states.[16]

Regulatory Model Assessment & Recommendations

The Framework

In an effort to standardize how appropriate regulatory models are determined for each profession (e.g. license, registry, no regulation, etc.), OPLR developed a framework which incorporates its statutory review criteria.[17] Appropriate models are determined principally by an evaluation of the potential for harm and related factors that may aggravate or mitigate the potential for harm. These factors include the availability of consumer choice, vulnerability of patients, and independence of practice. See Appendix 3.1 for potential regulatory models and the factors in OPLR's framework.

Potential for Harm

OPLR found the risk of severe and permanent harm that could be caused by a HIS to be quite

[13] Per R156-46a-302.5(3), to practice under indirect supervision, an intern must "have received a passing score on each section of the International Hearing Society (IHS) Practical Examination for Hearing Instrument Interns" and "have successfully completed the International Hearing Society (IHS) Distance Learning for Professionals in Hearing Health Sciences Course." The IHS distance learning course is self-directed and paced, taking on average around six months to complete. Applicants may take the IHS exam, at their discretion and whenever they feel prepared.
[14] Effective January 1, 2025, Nevada bill AB177 removed the requirement for hearing aid specialists to be NBC-HIS certified, making it one of three options to obtain licensure. Additionally, Alabama has levels of state licensure, which include a hearing aid apprentice, a licensed hearing aid specialist, and an NBC-HIS dispenser. While not required for a hearing aid apprentice or a licensed hearing aid specialist, Alabama requires that the NBC-HIS dispenser be both licensed by Alabama and hold the NBC-HIS board certification.
[15] NBC-HIS Requirements
[16] Other states typically require internship periods of six months to a year.
[17] Among other criteria, OPLR is required to evaluate "whether the regulation of the occupation is necessary to address a present, recognizable, and significant harm to the health, safety, or financial welfare of the public" and consider "potentially less burdensome alternatives to the… existing regulation". UCA 13-1b-302

low overall. A HIS performs some tests and procedures that carry some risk, but even these services are inherently safe and are associated with only rare safety issues. One of the more risky procedures includes performing earmold impressions which, if performed incorrectly, could result in negative downstream impacts. This procedure involves inserting an otoblock (a temporary plug inserted into the ear canal to protect the eardrum), using a syringe to insert impression material into a patient's ear canal and outer ear, waiting for the material to set, and removing the impression. The primary risk associated with this procedure is physical trauma to the ear canal or eardrum due to improper otoblock placement which can expose a patient's eardrum to the impression material. However, a HIS noted that as the hearing instrument technology evolves, this practice is being used less frequently. Additionally, there is a small risk of negative downstream effects from failure to identify more serious health conditions (like an acoustic neuroma)[18] causing hearing loss to further deteriorate. The Occupational Information Network (O*NET), developed under the sponsorship of the U.S. Department of Labor, estimates an occupation's consequence of error based on how serious the result would be if a mistake were made.[19] The consequence of error for a HIS is 34 out of 100, which is significantly lower than that of other healthcare professionals.[20]

There are some potential downstream effects of poor or delayed care. If a HIS provides inadequate care, a patient could have an incorrectly interpreted hearing test and an inadequately programmed hearing instrument. This could result in overamplification, which could further impair a patient's hearing. In rare cases, a HIS may fail to recognize concerns regarding more severe disease or disorders, thus not referring a patient to an audiologist or physician. This could result in a worsening of the condition.

Related Factors

Patients and consumers have a high degree of patient choice when determining which HIS to see, which further mitigates the potential for harm. Since many HISs work in settings such as retail hearing centers and outpatient healthcare facilities,[21] patients can analyze reviews of potential providers and schedule an appointment with the HIS of their choice. By having the ability to make informed choices, the risk of going to an unsafe or incompetent practitioner is mitigated.

Additionally, with a large segment of the HIS workforce employed by retail hearing centers,[22] there is an added level of oversight from the employer. Conversations with one of the largest retail employers of HISs revealed that there is extensive internal training in addition to state requirements. Furthermore, the retail employer described frequent quality control checks and reports that analyze the safety and effectiveness of their HISs. This type of extensive employer

[18] An acoustic neuroma is a non-cancerous tumor that develops on the main nerve (vestibular nerve) leading from the inner ear to the brain. Branches of the nerve directly affect balance and hearing. Pressure from this tumor can cause hearing loss, ringing in the ear, and trouble balancing.
[19] O*NET Consequence of Error
[20] O*NET scores for similar professions include audiologists (40) and licensed practical nurses (61).
[21] U.S. Bureau of Labor Statistics; Since Utah specific data is unavailable, OPLR relied on and extrapolated from national data.
[22] Ibid.

oversight reduces the potential for harm. However, some HISs are in private or group practice, making it important that the state still maintain some level of DOPL oversight for the profession.

For more details on OPLR's analysis of HISs according to the framework, see Appendix 3.2.

Recommendation: Shift the Regulatory Model for HIS from Licensure to Mandatory Certification

OPLR's analysis concluded that the potential for harm from a HIS is relatively low, thus warranting the use of a lower burden regulatory model, such as certification. However, there are some concerns about the downstream implications of receiving poor or improper care due to a lack of competence from a HIS. For this reason, OPLR recommends making the certification mandatory, rather than voluntary, to ensure a minimum level of education and training.

OPLR proposes shifting the regulatory model for HISs from the current licensure model to a mandatory certification model. Mandatory certification should require HISs to:

1. Certify with the Utah Division of Professional Licensing (DOPL) only once using entry requirements, without the need for renewal; and
2. Maintain either national certification *or* continuing education *or* a minimum number of hours of practice without lapse.

Mandatory certification would be similar to licensure in that a HIS would still be required to provide evidence of competence in an initial application to DOPL. Entry requirements would be maintained, although streamlined, as recommended below.

Mandatory certification differs from licensure in this case in one distinct way. HISs would not be required to complete a biennial renewal process, thus reducing the burden on providers and DOPL. In the absence of renewal, a HIS would still be required to maintain national certification, continuing education, or a minimum number of hours of practice without lapse to ensure ongoing competence. By maintaining one of these, a HIS would be required to stay current and accountable to the profession without requiring that s/he formally interact and pay a renewal fee to DOPL. As is the case in other regulated professions, the law could require that individuals maintain records proving compliance and provide them to DOPL if requested.

Regulatory Model Adjustments & Recommendations

After determining an appropriate regulatory model, OPLR's framework also evaluates whether adjustments should be made within a recommended model to address any material and existing safety and access issues affecting the Utah public and practitioners. Regulatory model adjustments may include changing entry qualifications, the scope of practice, unprofessional or unlawful conduct, and/or supervision and independence provisions (see Appendix 4.1).

Safety Issues

OPLR identified low levels of reported harm by HISs and HIS interns from available data. Using

DOPL complaint data from 2017 to 2022, there were a total of three substantiated complaints against a HIS, for a rate of 0.34 substantiated complaints per 100 annually.[23] None of the three substantiated complaints were related to client harm or endangerment; rather, they were related to aiding unlicensed practice, practicing beyond the scope of the license, and unauthorized practice.[24] Since these complaints do not relate to client harm, there was no evidence of HISs causing widespread, serious harm to the public.

Similarly, for HIS interns, there were two substantiated complaints from 2017 to 2022, for a rate of 0.4 substantiated complaints per 100 annually. The substantiated complaints did not include any complaints related to patient harm. Instead, complaints included unprofessional conduct and practice beyond the scope of the license.[25] Given the nature of complaints, HIS interns were not deemed to cause serious harm to the public.

Access

Patient access to HIS appears to be broadly sufficient in Utah, although data on this occupation is fairly sparse. Between 2014 and 2024, Utah experienced a 5.0% annualized growth in licensure of HIS, which outpaced population growth,[26] although HIS interns experienced a decline in licensure of 1.5% over the same period. Additionally, Utah's location quotient, or the share of HIS employment in Utah relative to the rest of the U.S., is 215% which indicates that Utah has a significantly higher rate of HISs compared to the U.S. generally.[27] Furthermore, audiologists offer an additional source for evaluating hearing loss and dispensing hearing aids.

The HIS profession is moderately accessible for potential practitioners, although OPLR's conversations with HISs revealed some overly burdensome requirements to enter the profession and confusion regarding requirements to become licensed.[28] Some features to become a HIS make the profession fairly accessible. Obtaining the high school or equivalent education requirement is relatively attainable. Additionally, ordering and studying the required IHS Distance Learning for Professionals in Hearing Health Sciences Course is available to anyone, with a relatively low financial commitment. However, Utah is the only state that requires the NBC-HIS board certification as an entry requirement for full licensure as a HIS. OPLR's conversations with HISs and employers and analysis of state licensing laws for HISs, suggest this is overly burdensome. For those who wish to pursue licensure through the education pathway, there are only two schools in the U.S. offering these programs.[29] The employment pathway is complex and not well described in statute and rule. Additionally, the employment pathway to licensure is reliant on the availability of supervising HISs.

[23] This rate is nearly the same as audiologists (0.32), registered nurses (0.30), and licensed practical nurses (0.36) but lower than physician assistants (0.56) and acupuncturists (0.82). However, OPLR is cautious when comparing complaint rates across professions, however, because professions engage in different services that contain different levels of risk for patients.
[24] See Appendix 4.2 for OPLR's analysis of DOPL complaints
[25] OPLR analysis of DOPL complaint data
[26] DOPL Licensing Data; According to Utah Department of Health and Human Services Indicator-Based Information System for Public Health, Utah experienced 1.8% annualized population growth between 2014 and 2024
[27] OPLR's analysis of U.S. Bureau of Labor Statistics data
[28] OPLR's conversations with HIS and HIS interns
[29] Become a Hearing Aid Specialist

Recommendation: Streamline the Pathway to become a HIS

OPLR recommends adjusting the examination requirement to become a HIS. To achieve this, OPLR recommends eliminating board certification by the NBC-HIS as an entry requirement. Instead, applicants should be allowed to submit evidence of passage of the IHS practical examination and the ILE examination in addition to completing six months of supervised practice as a HIS intern (rather than two years needed for the NBC-HIS). While OPLR recommends against requiring the NBC-HIS as the entry standard for becoming a HIS, OPLR does recommend that applicants may still use the NBC-HIS board certification as an option to qualify for certification, increasing flexibility. See Appendix 4.3 for more detail on the recommended training pathway to become a HIS.

This recommendation is based on the overly burdensome nature of obtaining the NBC-HIS board certification for an entry-level HIS. The NBC-HIS is a way to demonstrate highly advanced skills as a HIS and to be a "distinguishing mark for the highest standards of skill and knowledge"[30], rather than to demonstrate entry-level competence.

Recommendation: Allow Audiologists to Supervise HIS Interns

OPLR recommends improving and expanding mentorship opportunities for HIS interns by allowing audiologists to be mentors and supervisors for purposes of meeting certification requirements. Since audiologists are highly qualified and are trained to prescribe and dispense hearing instruments, adding more potential mentors and supervisors would expand the opportunities for HIS interns to enter the profession. Both supervising HISs and audiologists are required to hold certification in their respective field, be in good standing, and have practiced full-time for at least two years.

Rule Review

In accordance with Utah Code 13-1b-203(5), OPLR conducted an in-depth review of DOPL's hearing instrument specialist rules, found in R156-46a.

The rule review covered potential rule changes needed to:

1. address specific rules that may be either overly burdensome (e.g., for individuals seeking to practice a profession or given the potential risk to public safety from a profession, etc) or insufficient (e.g., to ensure safe practice);
2. address rules misaligned with statutory language;
3. clarify language and correct references to statute or other rules; or
4. support OPLR's recommendations.

OPLR's review of R156-41 found:

[30] NBC-HIS Certification

1. no overly burdensome rules.
2. no rules misaligned with statutory language.
3. two incorrect references to statute. These are outlined in Appendix 5.1.
4. new rules will need to be written to support the shift to mandatory certification. These rules include removing the license renewal requirements,[31] continuing education requirements,[32] and establishing a minimum number of hours of certified practice without lapse to maintain certification. Additional rules will need to be written to adjust the updated examination requirements and internship supervision requirements.[33]

[31] Renewal cycle is defined in R156-46a-303
[32] Continuing education is defined in R156-46a-304
[33] Examination requirements and internship supervision is defined in R156-46a-302.5

Appendix

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1. Context

1.1 General Research Methodology

OPLR's methodology combines qualitative and quantitative methods with robust stakeholder engagement. Methods include:

- Analyzing data from workforce surveys administered by the Department of Professional Licensing (DOPL) as part of licensure renewal
- Conducting quantitative analysis of DOPL licensee and complaint data and publicly available data from other state and federal government entities (e.g., DWS, HRSA)
- Reviewing academic literature and reports on a profession's practice, efficacy and safety
- Scanning education and credentialing requirements, programs and content
- Reviewing state occupational regulation policies across the U.S.
- Engaging with a wide range of stakeholders, including: Utah governments and agencies, industry organizations, researchers, practitioners, and business owners and employers within a variety of settings

2. Background

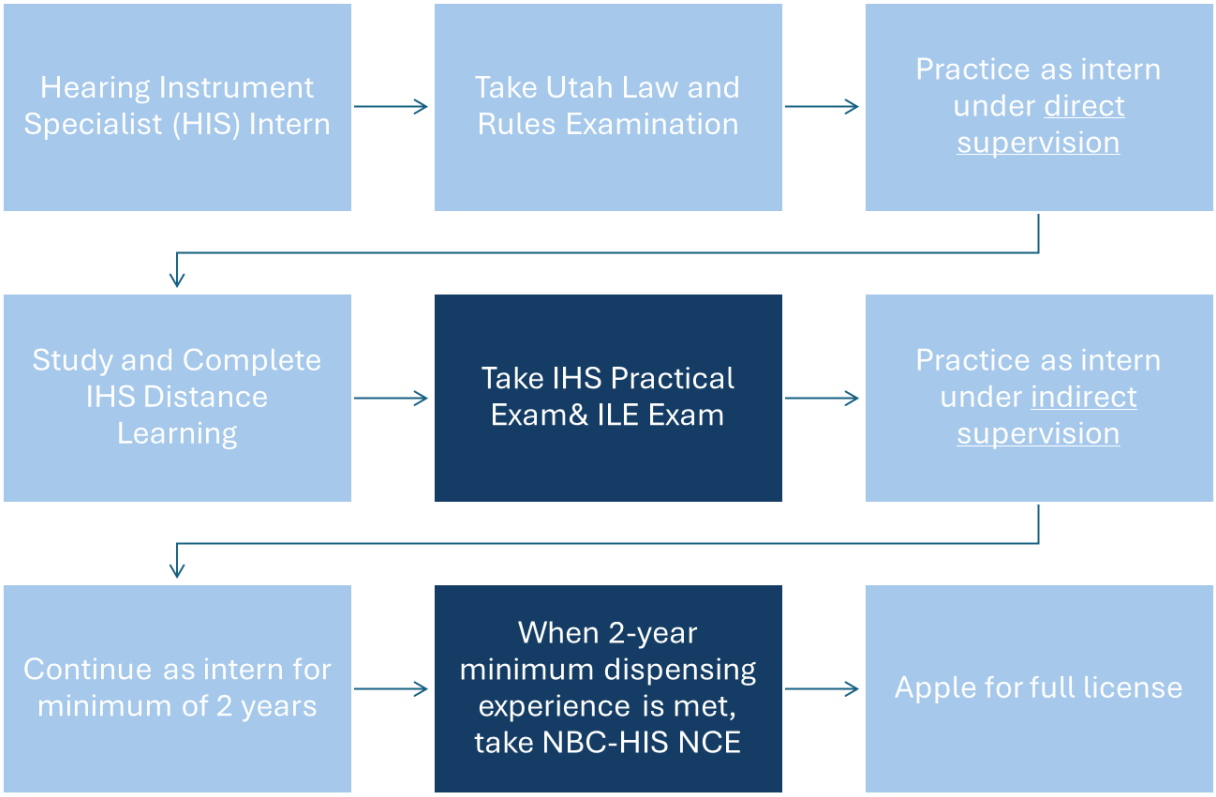
2.1 DOPL Licensee Data

OPLR used DOPL licensee data queried in January of 2025 to conduct analyses on the number of licensees per year, inflow and outflow of licensees, overlap of licenses, and time with license. The dataset included individuals first licensed after 1970 to those actively licensed as of January 2025. Each row in this dataset was a unique combination of individual and license type and contained information regarding when the license was issued, the status of the license, the date the status was last updated, and the sex and year of birth of the individual. OPLR estimated the number of licensees in each year by summing the number of unique individuals whose licenses were active during any point in each year. Additionally, OPLR excluded any individual with a null or incorrect value for their license issue date and license expiration date, as OPLR could not determine how long or for what years they were actively licensed. License counts may slightly underestimate the true number of licensees due to this, but the effect is fairly negligible given OPLR's use of the data to determine trends over time rather than estimate with precision for specific dates.

Between 2014-2024, the number of HISs licenses in Utah grew with a Compound Annual Growth Rate (CAGR) of 5.0%, which far outpaced the growth in the population of Utah during that period (1.8%).[34]

[34] Retrieved Mon, 03 November 2025 from the Utah Department of Health and Human Services, Indicator-Based Information System for Public Health website: https://ibis.utah.gov/ibisph-view/

2.2 Current Process to Become a Hearing Instrument Specialist in Utah



Source: OPLR analysis; IHS: International Hearing Society; ILE: International Licensing Examination for Hearing Healthcare Professionals; NBC-HIS: National Board for Certification in Hearing Instrument Sciences; NCE: National Competency Exam

3. Regulatory Model Assessment & Recommendation

3.1 Menu of Regulatory Models and Factors Considered in Framework

Please see **this working document**, OPLR's Occupational Regulation Framework, for a more detailed explanation of OPLR's approach to assessing occupational regulation and evaluating different regulatory models.[35]

3.2 Assessment of Hearing Instrument Specialists

The following table summarizes OPLR's analysis of HIS according to factors that OPLR determined should influence the appropriate regulatory model for an occupation. Factors that OPLR considered as particularly determinative in its assessment of HIS are highlighted in bold.

[35] The document is also available on OPLR's website in the "About OPLR" section, accessible here: https://oplr.utah.gov/about-oplr/

Model Assessment of Hearing Instrument Specialists	
Harm Factors	
Mechanism of Harm	**HISs perform routine tests to assess the hearing of adult patients for the sole purpose of identifying whether hearing loss would be improved by the use of hearing aids. The improper hearing assessment has relatively low consequences. There is some potential for financial harm.**
Severity, Permanence, and Likelihood of Harm	An untrained HIS would likely incorrectly prescribe or program a hearing aid for a patient. This could result in the patient's hearing not improving in the anticipated manner.
Consequence of Error	**34 out of 100***
Downstream Impact	**If a HIS provides poor care, a patient could have an incorrectly interpreted hearing test and an inadequately programmed hearing instrument. In rare cases, a HIS may fail to identify more severe disease or disorders, like cancers, which could result in downstream impacts.**
Consumer & Setting Factors	
Patient Vulnerability	**HISs work only with adult patients. Since HISs don't work with people under 18 years old but do treat elderly populations impacted by hearing loss, there is a limited degree of patient vulnerability.**
Frequency of Physical Touch	HISs do physically touch patients during examinations. However, this is limited to contact around the ears and head.
Frequency of Private Setting	Many HISs work in retail settings, with some frequency of private contact with patients. Others work in private or group practices, where there is potential for more private contact.
Information Asymmetry	HISs perform hearing tests and prescribe hearing instruments. A typical patient may not have the knowledge to evaluate the specific test performed by a HIS, but a patient would have the knowledge to know if their hearing improved as a result of being prescribed a hearing instrument.
Related factors	
Independence	HISs have a high degree of clinical independence.
Patient Choice	**Patients have a high degree of choice in their HIS. Patients have the ability to research and analyze reviews of a variety of HISs in their proximity. This may be somewhat limited for patients going to a retail hearing instrument center, since the exact practitioner cannot always be selected, so patients**

	would have to rely on reviews of the facility.
Information Availability	**Because of the high level of patient choice, there may be a high level of information available to patients regarding the quality of care.**
Level of Oversight	***Employers:*** **Depending on the practice setting, there are varying degrees of employer oversight. In major retail hearing instrument settings, the oversight is very high, but in private or group practice, the employer oversight may be lower.** *Private Bodies:* HISs can be board certified by the NBC-HIS. In Utah, this is currently mandatory to be licensed. Additionally, in retail hearing instrument centers, the level of oversight is high.

*O*Net Consequence of Error Ranking based on practitioner

4. Regulatory Model Adjustments & Recommendation

4.1 Possible Adjustments

Please see **this working document**, OPLR's Occupational Regulation Framework, for a more detailed explanation of how OPLR approaches whether adjustments should be made within a recommended regulatory model.[36]

4.2 DOPL Complaint Analysis

The Division of Professional Licensing (DOPL) receives complaints from individuals, other state agencies, co-workers, professional associations, and licensing boards. DOPL is required to "investigate unlicensed practice in regulated professions, acts or practices inconsistent with recognized standards of conduct, allegations of gross negligence or incompetence, and patterns of gross negligence or incompetence". Violations that meet the criteria for investigation are then prioritized and assigned to an investigator. DOPL may resolve investigations in a variety of ways, including: closing an investigation due to a lack of evidence; referring the case to another agency or to law enforcement if appropriate; carrying out informal or formal administrative sanctions or stipulated agreements; issuing a citation; or denying, suspending, or revoking an individual's license.

To analyze complaints sent to DOPL, OPLR used My License Office (MLO) to access closed complaints investigated by DOPL between 2017-2022 for all licenses/professions. This data contains information on the license name, the complaint type, and the disposition of the complaint, among many other data fields not relevant to OPLR's analysis. DOPL personnel helped code the complaint dispositions as either substantiated, unsubstantiated, or no jurisdiction. Substantiated complaints are those where a disposition includes some type of

[36] The document is also available on OPLR's website in the "About OPLR" section, accessible here: https://oplr.utah.gov/about-oplr/

disciplinary action, whether formal or informal (e.g., letter of concern, verbal warning, surrender of license). Unsubstantiated complaints have dispositions without a disciplinary action (e.g., dismissed, lack of evidence, unfounded). 'No jurisdiction' complaints are complaints that may or may not have basis, but DOPL was not able to take action on the case.

OPLR filtered complaints to exclude any likely duplicates and then used substantiated complaints to calculate the number of complaints per license type or profession. OPLR estimated the complaint rate for each license type by dividing the number of substantiated complaints by the number of unique individuals who held that license type over the same period.

Complaint Case Notes Analysis

A more detailed analysis of historical case notes was conducted on HIS complaints closed between 2017-2022. OPLR analysts read through case notes from all three complaints and for each complaint summarized the issue, noted whether or not client harm occurred or potentially occurred, and if harm was present, the type and severity.

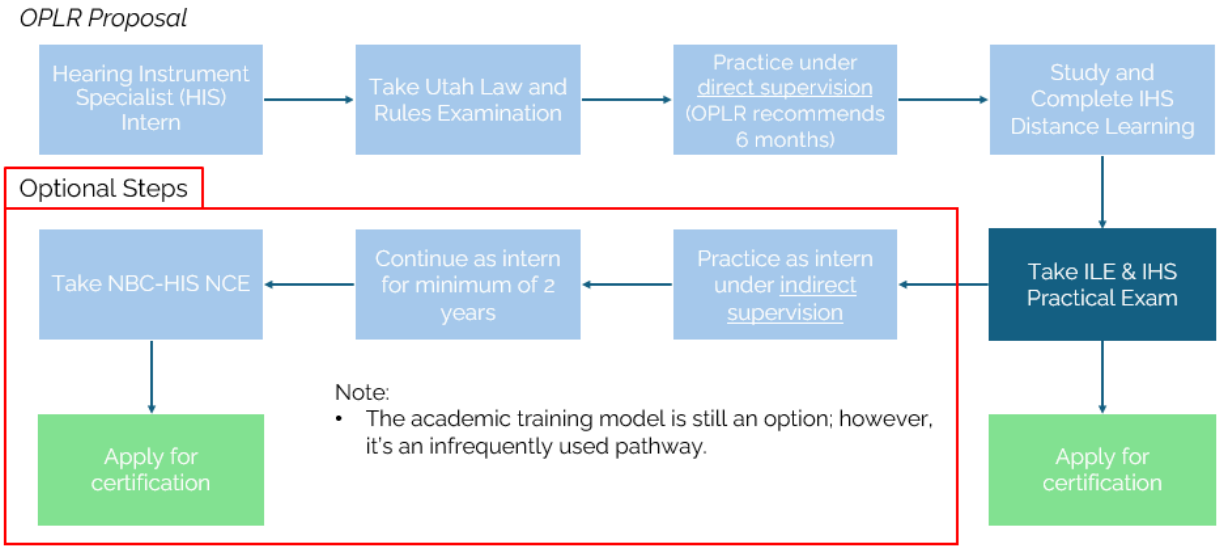
Limitations

There are significant limitations to this analysis, and the information collected should not be interpreted as a precise estimate of harm caused by HISs. DOPL data likely underestimates true harm, as many instances of harm may be handled in other ways (e.g., directly by employers), reported to other entities, or may never be reported. Additionally, some unsubstantiated complaints may have resulted in harm but the necessary evidence was not produced.

There could also be latent factors correlated with both the likelihood of complaint and the profession, systematically biasing the comparisons across professions. This is especially true in healthcare, as certain professions, by their nature, include a greater potential for harm and may generate more complaints. For example, surgeons have a higher likelihood of causing severe harm to a patient than HISs because surgery is inherently far riskier, not because surgeons are "less safe" or less competent than HISs.

For these reasons, OPLR uses DOPL complaint data as directionally informative, but avoids direct comparisons across professions wherever possible. Fine comparisons across professions are unwarranted and unsupported by these data.

4.3 Recommended Process for Becoming a Hearing Instrument Specialist



Source: OPLR analysis; IHS: International Hearing Society; ILE: International Licensing Examination for Hearing Healthcare Professionals; NBC-HIS: National Board for Certification in Hearing Instrument Sciences; NCE: National Competency Exam

5. Rule Review

5.1 Incorrect References

OPLR identified the following incorrect references in the Hearing Instrument Specialist Licensing Act Rule.

Rule	Incorrect Reference	Correct Reference
R156-46a-101(3)	R156-1-107	R156-1-107 was repealed in 2021
R156-46a-502a(3)(a)	Subsection 58-46-502(5)	Subsection 58-46a-502(5)

6. Stakeholder Engagement

6.1 OPLR Interview Series

OPLR relied heavily on stakeholder engagement and qualitative interview data, combined with OPLR's other analysis, to conduct this review and develop recommendations. OPLR engaged with HISs, HIS business owners and employers, HIS educators, industry associations, Utah legislators, and Utah and other state regulators. OPLR prioritized diversity of perspective and relevance to the industry in selecting stakeholders. Variety in practice setting was also prioritized.

Interviews were conducted via video conferencing using semi-structured interview methods; they were conducted one-on-one and with multiple members. Extensive notes were taken for all interviews.

OPLR conducted initial interviews to understand the HIS industry, determine the largest issues related to safety and access, and identify potential areas for change. OPLR engaged with stakeholders later in its review to test initial findings from analysis and preliminary recommendations. OPLR reflected on and synthesized feedback across multiple discussion sessions to develop clear and achievable evidence-based recommendations.

Limitations

This interview sample was not randomly selected and, therefore, is not completely representative. OPLR spoke to individuals most likely to represent the broad aims and concerns of their groups. Additionally, OPLR did not contact "consumers" of HIS (i.e. patients), so their perspectives were not incorporated into this review. Thus, the stakeholder engagement and findings from these interviews should not be understood to be fully representative of the views of all Utahns, HISs, HIS employers or clinic owners, or any other person, group, or population.

Note that stakeholders' views are not always reflected in OPLR's recommendations. OPLR is directed by Utah Code 13-1b-302 to apply specific review criteria. These can and do lead to recommendations that diverge from stakeholder preferences. A stakeholder's appearance here is not an endorsement of OPLR's recommendations as such.

Stakeholder Engagement Summary - Speech & Hearing Professions	
Government Stakeholders	
Utah Department of Commerce	**Margaret Busse**, Executive Director **Carolyn Dennis**, Deputy Director **Jacob Hart**, Deputy Director **Mark Steinagel**, Director, Division of Professional Licensing **Lisa Martin**, Bureau Manager, Division of Professional Licensing **Tracy Taylor**, Bureau Manager, Division of Professional Licensing **Lindsay Aagaard**, Licensing Specialist, Division of Professional Licensing **Brylee Vanderwarf**, Board Secretary, Division of Professional Licensing
Division of Professional Licensing (DOPL) Board	**Brooke Hammond**, Audiology Board Member, Speech-Language Pathology and Audiology Board **Lindsey Hardcastle**, Speech-Language Pathology Board Member, Speech-Language Pathology and Audiology Board **Robert Kraemer**, Speech-Language Pathology Board Member, Speech-Language Pathology and Audiology Board **Lauren Snyder**, Audiology Board Member, Speech-Language Pathology and Audiology Board
Utah Department of Health and	**Shandi Adamson**, Office Director, Division of Integrated

Human Services	Healthcare **Stephanie McVicar**, Program Manager, Early Hearing Detection and Intervention, Office of Children with Special Health Care Needs **Jenny Pedersen**, Coordinator, Children's Hearing Aid Program (CHAP), Office of Children With Special Health Care Needs **Jessie Rodriguez**, Health Program Manager, Division of Integrated Healthcare **Jim Stamos,** Director, Office of Healthcare Policy and Authorization at Utah Medicaid **Gregory Trollan**, Office Director, Division of Integrated Healthcare **Debi Walker,** Health Program Manager, Division of Integrated Healthcare **Shannon Wnek**, Audiologist, Early Hearing Detection and Intervention (EDHI), Office of Children with Special Health Care Needs
Utah State Board of Education	**Kristin Campbell**, Research Consultant **Megan Carlisle**, Educator License Equivalency Specialist **Jonathan Collins**, Research Consultant **Jordan DeHaan-Magalei**, Supporting Personnel and Preparation Coordinator **Kim Fratto**, Director of Special Education Programs **Maria Hawley**, Related Services Personnel Preparation Specialist **Malia Hite**, Executive Coordinator of Education Licensing **Lisa McLachlan**, Educator Preparation Coordinator
Industry Stakeholders	
Industry Associations	**Susan Adams**, Director of State Legislative & Regulatory Affairs, ASHA **Matt Hansen**, Executive Director, Homecare & Hospice Association of Utah **Kenyatta Jones Hunt**, Certification Program Director, National Board for Certification in Hearing Instrument Sciences **Katie Meyer**, Senior Director of Ethics, ASHA **Peter Mihalick**, Health Policy and Advocacy Director, International Hearing Society **McKenna Nobis**, President-Elect & SLP Clinician, Utah Speech-Language Hearing Association **Lee Robinson**, President, Utah Speech-Language Hearing Association; Professor, Brigham Young University Department of Communication Disorders **Christine Seitz**, Manager of Government Affairs, International Hearing Society **Allison Spangler**, President & CEO, Utah Health Care Association **Mary Stone**, Senior Certification Administrator, National Board for Certification in Hearing Instrument Sciences
Employers	**Jeffrey Elliott**, Optical/Hearing Regional Manager, Costco Wholesale **Joseph Kamerath**, Physical Medicine and Rehabilitation, Intermountain Health **Tammy Miller**, Director of Training for Hearing Aids, Costco Wholesale

Higher Education	**Sarah Hargus Ferguson**, Professor, University of Utah Department of Communication Sciences & Disorders **Mark Rasmussen**, Clinical Professor, University of Utah Department of Communication Sciences and Disorders **Teresa Ukrainetz**, Assistant Department Head & SLP Division Chair, Speech and Hearing Sciences, Utah State University
Subject-Matter Experts	
Academics, Researchers, & Clinicians	**Julie Barkmeier-Kraemer**, SLP Clinician & Professor, University of Utah Health **Jordan Bigler**, HIS, Pure Hearing **Joe Dansie**, AUD Clinician & Founder, Conditioned Play Innovations **Noah Hadley**, Clinical SLP, Copper Ridge Health Care - Skilled Nursing **Kate Johnson**, Clinical Audiologist, University of Utah Health **Harry Leibovich Sr.**, Audioprosthologist, Miracle-Ear Hearing Aid Center **Angela Menlove**, Clinical SLP, Intermountain Health **Kacee Muller**, Clinical SLP **Jessica Nelson**, Director of Treatment, Timpanogos Hearing & Tinnitus **Michael Page**, Audiologist **Neil Patel**, Professor and Otolaryngologist, University of Utah Health and Intermountain Health **Jo Puntil**, SLP Clinician & ASHA Fellow, St. George Regional Hospital/Intermountain Health **Katie Stone**, Professor, Brigham Young University Department of Communication Disorders **Katie Tonkovich**, Audiologist, Primary Children's Hospital

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